National Cancer Control Programme (NCCP) 00449 · clinician draft

FOLFIRI + bevacizumab

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

FOLFIRI + bevacizumab · active treatment cycle

Active medicines: Bevacizumab + Irinotecan + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally once	Before Day 1 chemotherapy Review QT risk, interactions, constipation and hepatic function.
Dexamethasone	8 mg orally once	Before Day 1 chemotherapy Reconcile all steroid prescriptions; do not duplicate.
Dexamethasone	8 mg orally once daily	Days 2 and 3 after Day 1 chemotherapy Confirm the prescribed delayed prophylaxis and re-anchor it if treatment is delayed.
Metoclopramide	10 mg orally up to three times daily as required	Rescue only; duration to be completed by prescriber Review renal function, neurological contraindications and interactions.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan if prescribed Avoid in glaucoma. Acute cholinergic support is separate from delayed-diarrhoea treatment.

Additional phase-specific support

- Take the prescribed prevention medicines at the planned times and keep rescue treatment available. Tell the team if nausea or constipation is not controlled; do not duplicate steroid or anti-sickness medicines.
- Irinotecan can cause immediate cramps or sweating during the infusion and delayed diarrhoea afterwards. These need different treatments. Ask for the written rescue and urgent-contact plan before starting.
- White-cell growth-factor injections are prescribed according to individual risk and previous complications. They are not automatically required for every patient or every cycle.
- Keep the agreed blood-pressure and urine checks. Report bleeding, poorly healing wounds, new swelling or severe headache promptly.
- National Cancer Control Programme (NCCP) delayed-diarrhoea rescue: loperamide 4 mg initially, then 2 mg every 2 hours until 12 hours after the last liquid stool; do not continue this high-dose schedule beyond 48 hours. Provide urgent escalation for fever, dehydration, pain or persistent diarrhoea. This is not the capecitabine-only rescue schedule.
- No routine bevacizumab hypersensitivity premedication unless previous reaction or a specific local plan. Review blood pressure, proteinuria, bleeding and wound healing separately from nausea prevention.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00449 v6a · NCCP NCIS antiemetics V8, Table 1; apply oral regimen guidance separately

Sources checked 2026-11-04 · SACTCheck v0.84.21 · Regimen-level drafts; prescriber and oncology-pharmacy verification required